**Audit report for Filey Care and Support LTD**
**DATE: 2nd, 3rd & 4th March 2026**

**Provider: Filey Care and Support Ltd**
**Location: Enfield, Greater London**
**Current CQC rating: Good**

HOMECARE SERVICE

# Filey Care and Support LTD

Overall: Requires improvement

Chase Green House, 42 Chase Side, Enfield, EN2 6NF 07984 818399
Provided and run by: Filey Care and Support Ltd

| OUTSTANDING OVER 87% | GOOD 63% - 87% | REQUIRES IMPROVEMENT 39% - 62% | INADEQUATE 25 – 38% |
|---|---|---|---|

| Citation Rating Opinion | | | |
|---|---|---|---|
| | Score | % | RATING |
| SAFE | 19 | 59 | Requires Improvement |
| EFFECTIVE | 15 | 63 | Good |
| CARING | 14 | 70 | Good |
| RESPONSIVE | 18 | 64 | Good |
| WELL-LED | 17 | 61 | Requires Improvement |
| | | | |
| SERVICE RATING | Requires Improvement | | |

*"This rating is based on evidence collated at the time of the visit to the service. The report is written in line with the relevant regulatory legislation and following current regulatory inspection methodology."*
This report is a summary of the key findings and suggested areas of improvement.

## Compliance Judgements

The judgements are made against the:

- Internal Quality Outcomes, Policies & Procedures
- Health and Social Care Act 2008 (Regulated Activities) Regulations 2014
- Judgements and recommendations related to H&S Legislation are made with reference to the following:
  - Management of Health and Safety at Work Regulations 1999
  - Workplace (Health, Safety and Welfare) Regulations 1992
  - Health and Safety at Work Act 1974

***Consultant:*** *Shorai Dzirambe*
***Date of Audit:*** *2nd, 3rd & 4th March 2026*
***Duration of Audit:*** *3 days on-site*
***Date of report:*** *23rd March 2026*
***Feedback sheets:*** *Electronic feedback*

## Detailed Findings

### Background

Filey Care and Support LTD is an independent domiciliary care provider delivering regulated activities to people living in their own homes as part of a supported living scheme. The service provides personal care and support to adults, including older people and individuals living with physical disabilities, long-term health conditions and dementia.

At the time of this mock inspection, the service was supporting 150 people, of whom 18 were receiving the regulated activity of personal care.

The mock inspection was conducted over three days, and we visited the head office where the regulated activity is managed and directed from, as well as several supported living schemes:

- Day one: Head Office and Church Street.
- Day two: 52 Long Lane and 19 Cissbury Ring Road
- Day three: Montague Gardens and Chase Road.

A review of care and support delivery and an assessment of care records and medicines management practices were conducted. People were also spoken with as part of this visit. Other records reviewed included governance systems, policies, staffing arrangements, training compliance, and quality assurance processes.

Care was delivered by a team of care workers overseen by a management structure which included a registered manager. The service operated a scheduling system to coordinate visits and ensure continuity of care. People's needs varied in complexity, with support ranging from assistance with personal hygiene and mobility to medication administration and meal preparation.

This inspection considered whether the service was operating in line with the Health and Social Care Act 2008 (Regulated Activities) Regulations 2014 and assessed performance against the Care Quality Commission's key questions: whether the service is Safe, Effective, Caring, Responsive and Well-led.

### Audit Findings

| SAFE | People are protected from abuse and avoidable harm | | | Requires Improvement | |
|---|---|---|---|---|---|
| | Total possible score for SAFE: 32 | | | SCORE | % |
| | | | | 19 | 59 |
| 4 = Evidence shows an exceptional standard | 3 = Evidence shows a good standard | 2 = Evidence shows some shortfalls | 1 = Evidence shows significant shortfalls | | |
| **Learning culture**<br>We have a proactive, positive safety culture based on openness and honesty, in which concerns are listened to, safety events are investigated and reported thoroughly, and lessons are learned to continually identify and embed good practices.<br>***Regulations 12, 16, 17, 20*** | | | | **Score (1-4): 3** | |
| | Incidents are reviewed to identify contributing factors and ensure appropriate actions are implemented to reduce risk and support continuous improvement. This approach also supports the duty of candour, with people's loved ones informed of incidents or areas of concern where appropriate. For example, following an incident | | | **No Actions Required** | |

| | | |
|---|---|---|
| | during a community music session, an individual was restricted from attending due to unpredictable behaviour that posed a potential risk in the group setting.<br><br>The incident review identified that overstimulation, unexpected physical contact and close proximity to others acted as triggers for agitation, particularly during transition periods at the end of the session. As a result, the individual's risk assessment was updated to include additional mitigation measures, such as clearer boundaries, avoidance of physical contact, and leaving activities earlier if signs of agitation are observed. Staff also held a one-to-one discussion with the individual to reinforce expectations around appropriate behaviour.<br><br>Learning from the review emphasised the importance of using clear non-contact social cues, maintaining appropriate personal space, and closely monitoring early signs of distress during community activities to enable preventive action. Attendance at group activities with unpredictable social contact now requires stricter risk management and careful consideration of suitability. | |
| **Safe systems, pathways and transitions**<br>We work with people and our partners to establish and maintain safe systems of care, in which safety is managed, monitored and assured. We ensure continuity of care, including when people move between different services.<br>***Regulations 12, 17, (9)*** | | **Score (1-4): 3** |
| | There are safe systems and processes in place, supported by a set of policies such as safeguarding, infection control, medication management, recruitment, positive behaviour support, restraint and intervention, and governance procedures. These policies provide staff with clear guidance to deliver care safely and consistently, helping ensure risks are appropriately managed and monitored.<br><br>The service uses Cura care planning software; however, not all services use this software. For example, Long Lane's care and support plans were paper-based (see Well-Led).<br><br>There is also evidence of continuity of care, with people supported by a consistent staff team where possible and initial assessments completed before admission to ensure the service can meet their needs. This indicates that risks are considered before transitions take place and that appropriate support planning is established from the outset.<br><br>The service demonstrates safe transitions and partnership working, particularly when individuals move between services, including transitions from children to adult services. Information sharing with families, previous providers and relevant professionals helps maintain continuity and reduces disruption for people receiving care. | **No Actions Required** |

| | | |
|---|---|---|
| | In addition, the service supports people in accessing external health and social care professionals, including physiotherapists, occupational therapists, psychiatrists, and psychologists. This multidisciplinary approach supports safe care pathways and ensures that individuals receive specialist input where required. | |
| **Safeguarding**<br>We work with people to understand what being safe means to them as well as with our partners on the best way to achieve this. We concentrate on improving people's lives while protecting their right to live in safety, free from bullying, harassment, abuse, discrimination, avoidable harm and neglect. We make sure we share concerns quickly and appropriately.<br>***Regulations 11, 12, 13, 9, (17, 20)*** | | **Score (1-4): 2** |
| | The safeguarding concerns are shared with relevant agencies when required. The service aims to balance people's right to independence with the need to keep them safe, ensuring safeguarding responsibilities are clearly understood by staff.<br><br>Several incidents were reported through appropriate channels and reviewed to identify risks and learning. These included occasions when individuals left the service without staff support and were reported missing to the police, only to be located shortly afterwards. Following these events, measures such as GPS monitoring, telecare equipment, and personalised lanyards were used to reduce risk, and staff received additional training to strengthen practice. Another incident involved excessively hot shower water, which resulted in a blister on a person's skin. This was reported, reviewed and highlighted the importance of monitoring water temperature and ensuring safety checks are consistently completed.<br><br>Safeguarding incidents also included situations in which individuals displayed behaviours of distress and posed a risk to themselves or others. In one case, an individual became physically aggressive during a community activity and struck another attendee. Staff intervened promptly, de-escalated the situation and "*removed*" the individual from the environment to prevent further harm. It was unclear how the person was "*removed*" from the environment. A manager commented that the person was verbally encouraged to move away and redirected to another area. This meant this record lacked clarity around how the incident was managed. As a result, the person was banned from this activity. The use of restrictions or bans on activities should be reviewed to ensure they are proportionate, time-limited and regularly reviewed.<br><br>The person's risk assessment was reviewed and updated with additional mitigation measures, including avoiding unexpected physical contact and closely monitoring behavioural triggers in community settings.<br><br>In another incident, a person became highly distressed and self-injurious during medication administration, requiring staff to use approved physical intervention techniques to prevent further harm | **Actions Required**<br>*Strengthen recording practices to ensure incident reports clearly describe what occurred, how staff intervened, and what actions were taken to keep people safe. Review the use of restrictions or bans from activities to ensure they are proportionate, time-limited and regularly reviewed.* |

| | | |
|---|---|---|
| | before medical support was sought. This incident highlighted the need for consistent debriefing and clear recording following restrictive interventions. Evidence was not always available to demonstrate that debriefs were conducted following an incident.<br><br>There was also an incident during a family visit where an individual became physically aggressive towards a child in the garden. Staff and family intervened quickly to prevent escalation, and appropriate follow-up actions were taken. Reviews identified that environmental stimulation, fatigue and insufficient awareness of behavioural triggers may have contributed. Learning from this event included the need for clearer communication with family members about risk management during visits and closer monitoring of early behavioural cues. | **Action Required**<br>*Introduce a structured post-incident review process to ensure that all safeguarding and other incidents are followed by a documented debrief with staff and, where appropriate, the person involved.* |
| **Involving people to manage risk**<br>We work with people to understand and manage risks by thinking holistically, so that care meets their needs in a way that is safe, supportive, and enables them to do the things that matter to them.<br>***Regulations 9, 11, 12, (10)*** | | **Score (1-4): 2** |
| | Care and support records show that people's preferences, communication needs and daily routines are considered when planning support. Some people have communication passports, health passports and "About Me" profiles that describe their likes, dislikes and support needs. For example, information about routines, preferred activities and communication styles is recorded to guide staff in providing consistent support. In some cases, visual or pictorial materials have been used to help people understand changes or events affecting them, demonstrating an attempt to adapt communication to individual needs.<br><br>However, records also show inconsistencies in the involvement of people in care planning and risk management. Some care plans contain generic information or copied clinical explanations rather than personalised guidance for staff.<br><br>There are also examples where documents were not fully accessible, such as tenancy agreements in small font or service user guides without pictorial formats for people who may benefit from visual information (see Responsive).<br><br>Some consent forms were signed by relatives without the appropriate legal authority. In other cases, the person themselves signed despite concerns about capacity, indicating that Mental Capacity Act processes were not always applied consistently (see Effective).<br><br>Risk management documentation also varied in quality. While some risk assessments described behaviours, triggers and strategies for de-escalation, others were incomplete, outdated or not aligned with support plans. For example, behaviour support plans were blank or under review in some cases, and keyworker sessions or | **Action Required**<br>*Review and update all care plans and risk assessments to ensure they are fully personalised, accurate and reflective of the person's current needs.*<br><br>**Action Required**<br>*All risk assessments must consistently and clearly describe triggers, early warning signs, proactive strategies and reactive* |

| | | |
|---|---|---|
| | daily records did not always demonstrate how individuals were actively involved in making choices about their care or activities. These require regular reviews in partnership with Positive Behaviour Support (PBS) specialists, where required.<br><br>In other situations, care plans contained conflicting information, such as differing levels of support required for personal care or repositioning. Records of staff completing tasks, such as skin integrity checks or physiotherapy exercises, were not always documented, making it difficult to confirm that planned support is consistently delivered.<br><br>Records indicate that one person in Montague Gardens is supported with a range of health and social care needs, including dementia, learning disability, mobility difficulties and dysphagia risk. While staff demonstrated awareness of the person's needs, care plans and risk assessments do not consistently demonstrate how the person and their family are involved in planning, reviewing, and agreeing to care and risk management strategies. The person has minimal verbal communication and dementia, which increases the need for clear documentation showing how staff support communication and involvement in decision-making.<br><br>Several care plans require improvement to ensure guidance is accurate and sufficiently detailed to support safe care. The dysphagia plan contains conflicting information about food texture and refers to puréed diets despite the person being on an IDDSI Level 6 diet. The plan also lacks clear guidance on recognising signs of aspiration and the steps staff should take in the event of choking. This is particularly important as the person has a known tendency to overfill their mouth and eat quickly, and requires full supervision during meals.<br><br>Risk documentation also requires further clarity. A recent incident identified an increased risk of wandering, and an electronic monitoring device was introduced; however, the plan does not clearly state that the device must be taken whenever the person leaves the service. The daily support plan outlines routines but does not reference the funded one-to-one hours or the purpose of this support. In addition, the activities section is incomplete despite evidence that the person enjoys music, dancing and community activities.<br><br>Other documentation inconsistencies were identified. The skin integrity care plan refers to a different risk assessment tool than the one currently used by the service, which may confuse staff.<br><br>Evidence that the person and their family were educated or involved in aspects of care planning, such as skin care management, was not clearly recorded. | *interventions to reduce the likelihood of escalation.*<br>*Ensure all behaviour support plans are completed and regularly reviewed in partnership with Positive Behaviour Support (PBS) specialists where required.*<br>*Address inconsistencies in care documentation by ensuring support levels are clearly defined and aligned across all plans, including personal care, manual handling, and repositioning requirements.*<br>*Review and update care plans and risk assessments to clearly evidence the involvement of the person and family in care planning and review, and to document how staff support communication and involvement in decision-making.*<br><br>*Ensure the dysphagia care plan accurately reflects the current diet texture level and remove incorrect references to puréed diets. Include clear guidance on recognising aspiration and choking risks, and on the escalation procedure staff must follow (Montague Gardens).*<br>*Where people supported show an increased risk of wandering, ensure the care plan clearly states that the monitoring device must be with the person when they leave their home.*<br>*Ensure all sections of the care plan are completed to provide optimum care outcomes, such as the activities section.*<br>*Skin integrity risk assessment tools to coincide with what the service uses.*<br><br>*Evidence the involvement of the people supported and their families in their care planning.* |

| | | |
|---|---|---|
| | There was evidence that people receive support from health professionals, including physiotherapy, GP reviews, dental appointments and specialist input where required. However, some professional recommendations were not fully reflected in care plans or regularly reviewed (see Effective).<br><br>Activity records also indicated that many activities were structured by staff rather than clearly demonstrating that people had been offered meaningful choices or opportunities for community participation. Ensuring that people are actively involved in decisions about their daily lives, including activities, routines and health care, would further strengthen person-centred risk management.<br><br>Some care plans lacked important details, such as guidance for epilepsy, including seizure type, triggers, early warning signs, SUDEP awareness, etc.<br><br>In addition to the above, to make improvements, the service should:<br>• Ensure staff sign records to indicate that they have read and understood that people's plans are always up to date. For example, the signature list on one individual's file showed that it was last signed in June 2024.<br>• Ensure that there is evidence that information such as tenancy agreements and service user guides is explained to individuals and evidence of this is kept.<br>• Ensure the support needs detailed in support plans are reflected in daily records, for example, two individuals' support plan states that they should be offered two activity options; however, the daily records did not reflect that choice was being given. Another person's plan stated that they needed their skin checked daily; however, records did not show that this was occurring.<br>• Ensure that records always guide people, and they should be supported with dignity. For example, some plans describe people as 'stubborn'.<br>• Ensure keyworker sessions are happening as per the support plan, for example, two individuals' plans stated the keyworker sessions should happen weekly, but this was not taking place as planned.<br>• Ensure risk assessments and support plans are reviewed regularly, e.g., some plans were last reviewed in February 2025.<br>• Ensure that care and support plans are in place, only where a need is identified. For example, one individual had a substance misuse plan when there were no concerns regarding substance misuse.<br>• Where people are living with diabetes, ensure that the plans include signs of hypo and hyperglycaemia, including when to call for emergency services. | **Actions Required**<br>*Ensure that people are actively involved in decisions about their daily lives, e.g., activities, routines and healthcare, to further strengthen person-centred risk management.*<br>*Update all care plans to include clear and specific guidance for staff, e.g., epilepsy seizure type, triggers, early warning signs, SUDEP awareness, etc.*<br><br>*Ensure staff sign records to indicate that they have read and understood that people's plans are always up to date.*<br>*Ensure that there is evidence that information such as tenancy agreements and service user guides is explained to individuals, and evidence of this is kept.*<br>*Ensure the support needs detailed in support plans are reflected in daily records, e.g., daily records should reflect that a choice was being given.*<br>*Ensure records always guide people to be supported with dignity, e.g., describing people as 'stubborn'.*<br>*Ensure keyworker sessions are happening as per their support plan.*<br>*Ensure risk assessments and support plans are reviewed regularly.*<br>*Ensure that care and support plans are in place, only where a need is identified.*<br><br>*Diabetes plans must include signs of hypo- and hyperglycaemia and actions staff must take in an emergency.* |

| | | |
|---|---|---|
| | • One individual (SuM) had a history of constipation, and the GP advised on 16/2/2026 that she needed to be supported to minimise constipation by trying a laxative. The advice included continuing with adequate fluids and high fibre; however, there was no specific constipation plan, and neither was there an escalation plan, etc. The individual has haemorrhoids; however, the support plans did not reflect the support she received in this area. There was limited evidence that her bowel and fluid status were being monitored. | **Action Required**<br>*Ensure bowels and fluids are monitored correctly.* |
| | There was evidence that people receive support from health professionals, including physiotherapy, GP reviews, dental appointments and specialist input where required. However, some professional recommendations were not fully reflected in care plans or regularly reviewed. | **Action Required**<br>*Ensure professional recommendations are fully reflected in care plans and regularly reviewed.* |
| **Safe and effective staffing**<br>We make sure there are enough qualified, skilled and experienced people, who receive effective support, supervision and development. They work together effectively to provide safe care that meets people's individual needs.<br>***Regulations 12, 18, 19*** | | **Score (1-4): 2** |
| | Across the services, staffing arrangements are generally in place to support care, but gaps in recruitment records were observed. For example, there were some unexplained employment gaps for 3 out of 3 staff members, and references were not always verified. The service is required to review and update all staff recruitment records to ensure complete documentation, including verified employment history, references, interview notes, contracts, terms and conditions, and Right to Work checks.<br><br>In addition, all references for staff, including overseas employment, should be verified and filed, and DBS checks require confirmation that they have been obtained, recorded, and are valid. | **Actions Required**<br>*Review and update all staff recruitment records to ensure complete documentation, including verified employment history, references, interview notes, contracts, terms and conditions, and Right to Work checks.*<br>*Verify and file all staff references, including overseas employment, and confirm that DBS checks have been obtained, recorded, and are valid.* |
| | The REHABILITATION OF OFFENDERS ACT 1974 - NOTICE TO OFFENDERS on the application form states, *"Do you have any convictions, cautions, reprimands or final warnings that are not protected as defined by the Rehabilitation of Offenders Act 1974 (Exceptions) Order 1975 as amended in 2013? The amendments to the Exceptions Order 1975 (2013) provide that certain convictions and cautions are protected and not subject to disclosure to employers, and cannot be taken into account. Any information should be given on a separate sheet of paper and sent with this application form. This information will be treated as confidential and will not necessarily preclude you from employment."*<br><br>This statement does not meet the required standard because:<br>• It uses outdated terminology. The phrase "reprimands and final warnings" relates to the old youth disposal system (pre-2013). These have now been replaced by youth | **Action Required**<br>*Review and reword the statement on the application form about the rehabilitation of offenders.* |

| | | |
|---|---|---|
| | cautions and conditional cautions. While the older terminology is not wrong, it should be updated for clarity.<br>• Lack of clarity about exemption: It does not explicitly state that the role is exempt from the Rehabilitation of Offenders Act 1974 under the Exceptions Order.<br>• There is no reference to "filtered" offences: Best practice wording refers to "protected (filtered)" offences as defined by DBS guidance.<br>• No clear statement about DBS check requirement: No reference to confidentiality in line with data protection legislation<br><br>The service currently has two WTE-equivalent support worker vacancies.<br><br>Staff interviews were not always conducted by two interviewers, and individuals' names were not always written in full.<br><br>Staff spoken with reported that they had received adequate training to support individuals effectively. The training matrix demonstrated that staff had completed a range of mandatory and specialist training, including moving and handling, British Sign Language (BSL), breakaway techniques, epilepsy management and administration of rescue medication, and dysphagia. However, some discrepancies were identified:<br><br>• Staff stated they had completed additional training, such as Oliver McGowan Tier 1, which was not reflected in the training matrix. The staff members need to be trained for Tier 2.<br>• Furthermore, training in areas such as trauma-informed care and mental health was not recorded, despite the service supporting individuals with these specific needs.<br><br>Staff supervision had taken place, and staff reported that they were supported and had supervision and probation. One staff member commented, "*I have worked in care before, but this company is very good. I am happy at work, the money is good, and I enjoy my work, and I can't ask for more".*<br><br>Staff meetings are held to review incidents, reinforce professional boundaries, uniform standards, and communication protocols, and daily delegation forms allocate tasks and record completion. Minimum staffing levels are maintained, with sufficient cover during day and night shifts. Observations and spot checks of staff practice are conducted, and communication books are used to share people's needs and daily activities.<br><br>On-call guidelines are in place, and staff are reminded of how to access the out-of-hours on-call manager during meetings. | **Actions Required**<br>*Staff interviews were not always conducted by two interviewers, and individuals' names were not always written in full.*<br><br>*Review and update the training matrix to ensure all completed training (e.g., Oliver McGowan Tier 1 & 2) is accurately recorded. Identify and address gaps in training records, particularly in trauma-informed care and mental health, to reflect the needs of individuals supported.*<br>. |

| **Safe environments**<br>We detect and control potential risks in the care environment. We make sure that the equipment, facilities and technology support the delivery of safe care.<br>***Regulations 12, 15, 17*** | | **Score (1-4): 2** |
|---|---|---|
| | The service aims to maintain safe environments across its properties, including Chase Road, 19 Cissbury, 52 Long Lane, and Church Street.<br><br>**<u>Chase Road</u>**<br>At Chase Road, routine maintenance, such as lift servicing and pest control, has been completed, and PAT testing was carried out in August 2025. However, key documentation, including the Electrical Installation Condition Report (EICR) and external fire risk assessment, was not available at the time of inspection, limiting assurance that all actions have been completed.<br><br>**<u>19 Cissbury</u>**<br>At 19 Cissbury, the service primarily supports people who do not require regulated care. Health and safety checks are completed regularly, including emergency lighting, fire extinguisher inspections, weekly first aid kit checks, and weekly cleaning of kettles, washing machines, and showerheads.<br><br>Fire drills were carried out on 28 February 2026, and external fire risk assessment actions have been signed off. Some improvements were identified, such as updating the fire emergency plan to include a direct telephone contact for the fire brigade during emergencies. PAT testing, gas servicing, asbestos surveys, and EICR are up to date, and routine medication cabinet and fridge/freezer temperature checks are maintained.<br><br>**<u>52 Long Lane</u>**<br>At 52 Long Lane, several environmental risks were identified during maintenance works, including new flooring installation without a risk assessment and obstacles affecting people with mobility equipment. The environment was very dusty, noisy and posed as a trip hazard. Staff were observed supporting one individual who uses a wheelchair to go to her room with difficulty due to the work that was taking place.<br><br>Fire call points lacked signage, and some staff and volunteer risk assessments were overdue for review.<br><br>Each person has a PEEP in large font, and people were able to demonstrate understanding of fire procedures. Fire extinguishers, blankets, and hoists were serviced on schedule, and weekly checks of fire escape routes, alarms, doors, and emergency lighting are conducted.<br><br>However, some fire panel servicing and external fire risk assessment follow-up were overdue, and fire drill documentation | **Action Required**<br>*Obtain and file all missing documentation, including Electrical Installation Condition Reports, and ensure external fire risk assessments are accessible with actions clearly recorded as completed.*<br><br>**Actions Required**<br>*Review and update all fire emergency plans to include direct telephone contacts for the fire brigade and any additional site-specific risks.*<br><br>*Complete risk assessments for any ongoing maintenance work, including flooring installation at 52 Long Lane, considering obstacles, noise, and impact on people with mobility equipment.*<br><br>*Install signage for fire call points. Ensure all overdue staff and volunteer risk assessments are reviewed and updated in accordance with the stated schedule.* |

| | | |
|---|---|---|
| | did not consistently capture participation or relevant details such as actions taken, response times, and learning points. Duplicate forms should be replaced with a single robust template. Hot water temperature checks, washing machine and oven cleaning, and monthly hoist checks are completed.<br><br>**<u>Church Street</u>**<br>At Church Street, health and safety, infection prevention, and fire audits are completed monthly. COSHH data sheets are in place, and PAT testing and emergency lighting are up to date. Fire extinguisher checks, weekly fire escape inspections, and fire alarm tests are maintained. The gas service was due in November 2025. The external fire risk assessment from March 2025 requires evidence that actions have been completed, including confirmation of the fire assembly point. Fire panel servicing records and EICR were not available, and fire drill documentation lacks participant names and complete details.<br><br>Business continuity plans require expansion to include risks such as terrorism, fuel shortages, and extreme weather. | *Servicing and inspection of fire panels at Church Street and 52 Long Lane should be completed immediately, with records filed and reviewed.*<br>*Standardise fire drill documentation across all sites with a single robust template.*<br><br>*Bring all health and safety documentation for Church Street up-to-date, including evidence of completed actions in the FRA, gas and fire panel servicing records, and EICR, etc.*<br><br>*Business continuity plans require expansion to include risks such as terrorism, fuel shortages, and extreme weather.* |
| **Infection prevention and control**<br>We assess and manage infection risk. We detect and control the risk of it spreading and share any concerns with appropriate agencies promptly.<br>***Regulations 12, 15, (17)*** | | **Score (1-4): 3** |
| | Generally, across all services, infection prevention and control measures are in place to minimise the risk of infection and ensure safe care.<br><br>Cleanliness and hygiene standards are maintained for both the environment and the equipment, with regular cleaning schedules in place.<br><br>Staff and people using the services follow personal hygiene protocols, and food hygiene is managed according to policy.<br><br>Personal protective equipment (PPE) is accessible to staff and used appropriately during care and support.<br><br>Policies covering immunisation and management of outbreaks are in place. The policies guide staff to ensure that any incidents are reported promptly to the appropriate agencies. Policies for managing infectious diseases are current, and staff are trained to follow these protocols consistently, supporting a safe and hygienic environment for people living in the services. | **No Actions Required** |

| **Medicines optimisation**<br>We make sure that medicines and treatments are safe and meet people's needs, capacities and preferences by enabling them to be involved in planning, including when changes happen.<br>***Regulations 9, 12, (11)*** | | **Score (1-4): 2** |
|---|---|---|
| | Across the service, medicines are generally managed safely, with medication administration records (MARs) maintained and systems in place for monitoring stock levels. This includes recording medicines received and returning expired medications to the pharmacy.<br><br>Monthly medication audits are undertaken, and staff medication competencies are assessed regularly.<br><br>Temperature monitoring of medication storage areas is also completed, although action taken when readings fall outside recommended ranges was not always clearly documented.<br><br>However, several areas require improvement to ensure medicines are administered safely, consistently and in line with best practice.<br><br>When individuals were prescribed variable doses, there was insufficient guidance for staff on when to administer higher or lower doses. In addition, the exact dose given was not always recorded on the MAR chart. This was observed for medicines such as paracetamol, Beconase nasal spray, diazepam (PRN) and macrogol prescribed as a regular medicine. The absence of clear clinical indicators and accurate recording increases the risk of inconsistent administration.<br><br>The timing of medicine administration also required review. For one individual in Montague Gardens, medication that was prescribed for administration at 8:00am was administered at 2:00pm on 25 February. Records also showed that this person's morning medicines were sometimes administered between 9:00am and 10:30am rather than at the prescribed time. The service should seek clinical advice to clarify the acceptable administration window for these medicines to ensure safe and consistent practice.<br><br>In another example, documentation did not always align with prescribing records. One individual's care plan stated that paracetamol was prescribed as required; however, this medication was not recorded on the MAR chart. The same individual had a prescription for lorazepam, which was also not reflected on the MAR chart. Management advised that this medicine had not been required for some time; however, if the medicine remains prescribed, it should be accurately reflected in medication administration records.<br><br>PRN (as required) medications were in place; however, some protocols lacked sufficient detail. In several cases, guidance did not clearly specify the clinical indications for use, timing between | **Actions Required**<br>*Ensure temperature-monitoring records clearly document actions taken when storage temperatures fall outside recommended ranges.*<br><br>*Ensure MAR charts clearly record the exact dose administered, particularly where variable dosing is prescribed.*<br><br>**Actions Required**<br>*Seek clinical advice to clarify the acceptable administration window for medicines to ensure safe and consistent practice (Montague Gardens).*<br><br>*Medication documentation is required to align with prescribing records; e.g., paracetamol and lorazepam are not recorded/ reflected in the MAR chart.*<br><br>*PRN (as required) protocols did not contain sufficient detail or* |

| | | |
|---|---|---|
| | doses, or the triggers for administration. Where individuals had more than one medicine for the same condition, the protocol did not always clarify which medicine should be used as the first-line treatment. This lack of clarity may lead to inconsistent decision-making by staff. | *clear guidance between different medicines.* |

| EFFECTIVE | People's care, treatment, and support achieve good outcomes, promote a good quality of life, and are based on the best available evidence. | Good | |
|---|---|---|---|
| | Total possible score for EFFECTIVE: 24 | SCORE | % |
| | | 15 | 63 |

| 4 = Evidence shows an exceptional standard | 3 = Evidence shows a good standard | 2 = Evidence shows some shortfalls | 1 = Evidence shows significant shortfalls |
|---|---|---|---|

| | | |
|---|---|---|
| **Assessing needs**<br>We maximise the effectiveness of people's care and treatment by assessing and reviewing their health, care, wellbeing and communication needs with them.<br>***Regulations 9, 12, (10, 11, 17)*** | | **Score (1 – 4): 3** |
| | Across the services, people's needs are assessed to ensure care and treatment are person-centred. Assessments take into account health, well-being, communication, and accessibility requirements, including the use of mobility equipment to support independence. Technology, such as tablets and internet access, is available, and individuals reported that staff understand their needs.<br><br>Care and support plans are developed from these assessments and are reviewed regularly to reflect changes in health or preferences.<br><br>Clinical assessment tools, such as Waterlow and MUST, were used to identify people at risk of developing pressure ulcers and malnutrition.<br><br>People and their loved ones were involved in the assessments as appropriate.<br><br>Clients commented that staff work with them, which promotes choice and dignity. | **No Actions Required** |
| **Delivering evidence-based care and treatment**<br>We plan and deliver people's care and treatment with them, including what is important and matters to them. We do this in line with legislation and current evidence-based good practice and standards.<br>***Regulations 9, 10, 12, 14, 17, (11)*** | | **Score (1 – 4): 3** |
| | Care and treatment are planned and delivered with a focus on what matters to each individual. Staff follow recognised standards and policies to ensure safe, personalised care, including nutrition and hydration, supported by choice and assistance as required.<br><br>Clinical reviews are conducted to monitor health and well-being, but there are inconsistencies in the use of assessment tools; for example, one individual's care plan referenced the Braden tool for | **Action Required**<br>*Use clinical assessment tools consistently to guide care* |

| | | |
|---|---|---|
| | pressure area risk, yet the service routinely used the Waterlow scale.<br><br>Care is delivered taking account of people's preferences, promoting independence, and supporting engagement in daily activities. | *interventions and monitor outcomes.* |
| **How staff, teams, and services work together**<br>We work effectively across teams and services to support people. We make sure they only need to tell their story once by sharing their assessment of needs when they move between different services.<br>***Regulations 9, 12, (17)*** | | **Score (1 – 4): 3** |
| | The staff teams and external professionals work collaboratively to support people effectively and ensure continuity of care. Information about people's needs is shared on a need-to-know basis, supporting coordinated care while maintaining confidentiality. People spoken with confirmed they were kept informed about aspects of their care and support, which helped them understand decisions and feel involved. Before moving into the services, individuals were given opportunities to visit where possible, supporting smoother transitions and allowing them to become familiar with the environment and staff team.<br><br>Care coordination is supported through regular team meetings, handovers, and the use of communication books, which enable staff to share important information about people's needs, health changes, and daily activities. This helps ensure staff are aware of current risks, preferences, and support requirements, reducing the need for people to repeat their information multiple times. Delegation of tasks is managed within the staff team according to roles and competencies, supporting safe and consistent care delivery.<br><br>Transitions between services are supported through assessments and information sharing to ensure that the receiving service understands the person's needs and can provide appropriate support. | **No Actions Required** |
| **Supporting people to live healthier lives**<br>We support people to manage their health and wellbeing so they can maximise their independence, choice and control. We support them to live healthier lives and where possible, reduce their future needs for care and support.<br>***Regulations 9, 12, (9A, 10, 11)*** | | **Score (1 – 4): 2** |
| | People are supported to manage their health and wellbeing to maximise independence, choice, and control. Staff knew and understood people's needs well enough to identify changes in them. For example, on the day of the visit at 52 Long Lane, staff identified that one individual was showing early signs of distress and intervened by explaining the day's activity, including that the person would be supported to go out. This practice was observed across all the services visited during this inspection.<br><br>Individuals are supported in accessing healthcare professionals, such as GPs, dentists, and other specialists, to maintain their | |

| | | |
|---|---|---|
| | physical and mental health. Staff also promote healthier lifestyles by encouraging balanced nutrition, supporting people to remain physically active, and facilitating engagement in activities that promote wellbeing.<br><br>People are encouraged to take part in daily activities and community engagement that support physical movement and social interaction, helping maintain both physical and emotional health. However, in Long Lane, daily records reflected that activities were mainly home-based. The management team commented that this was due to staff not fully recording the support they provided. This meant the service did not always demonstrate that people were supported to participate in activities as per their plan.<br><br>In addition, staff training records indicated oral health assessment training had not been completed, and care plans did not reference the requirement for pre-medication before certain healthcare appointments.<br><br>Staff observe individuals for signs of deterioration in health and ensure concerns are escalated appropriately so that additional support or clinical input can be arranged. This proactive approach helps people maintain their independence while ensuring their health needs are met in a timely, coordinated manner. | **Action Required**<br>*Encourage participation in activities that promote physical movement, mental well-being, and social engagement, and ensure records reflect the support provided.*<br>*Oral health assessment training has not been completed, and care plans did not reference the requirement for pre-medication before certain healthcare appointments.* |
| **Monitoring and improving outcomes**<br>We routinely monitor people's care and treatment to continuously improve it. We ensure that outcomes are positive and consistent, and that they meet both clinical expectations and people's expectations.<br>***Regulations 12, 17, (9)*** | | **Score (1 – 4): 2** |
| | Systems are in place to monitor people's care and treatment through care plan reviews, incident monitoring, medication audits, and health checks. Outcomes are supported through access to healthcare professionals, such as GPs, psychiatrists, psychologists, physiotherapists, and occupational therapists. Staff monitor people for changes in health or behaviour and escalate concerns where required.<br><br>Medication audits are conducted monthly (where information was accessible), and competency assessments for medication administration are completed to support safe clinical practice. However, some inconsistencies were identified in clinical monitoring, such as unclear PRN medication protocols, variable medication administration times, and inconsistencies in the use of clinical assessment tools, for example, referencing the Braden tool in a care plan. In contrast, the service uses the Waterlow scale (see Safe).<br><br>Quality of life outcomes are promoted through person-centred support that encourages independence, use of mobility equipment, access to technology, such as tablets and the internet, and participation in community activities. People confirmed that staff | **No Actions Required** |

| | | |
|---|---|---|
| | understood their needs and supported them with daily living tasks and activities. Individuals are supported to access community resources, healthcare appointments, and social activities that contribute to their well-being. Opportunities to visit services before moving in were also offered where possible, supporting smoother transitions and improved experiences.<br><br>Some records, such as fire drill documentation and risk assessments, do not always capture all relevant information or are not consistently reviewed within required timescales. These gaps reduce the service's ability to clearly demonstrate how monitoring leads to measurable improvements in care quality and people's quality of life (see Safe). | |
| **Consent to care and treatment**<br>We tell people about their rights around consent and respect these when we deliver person-centred care and treatment.<br>***Regulation 11*** | | **Score (1 – 4): 2** |
| | There were policies and procedures to ensure people's rights around consent are respected and that care and treatment are delivered in line with the Mental Capacity Act 2005 and associated best practice. Mental capacity assessments (MCA) are completed for a range of decisions, including medical treatment, medication, finances, photographs, communal living arrangements, relationships, and restrictions within the environment. When individuals are assessed as lacking capacity, best-interest decisions are documented and involve staff and relevant parties to support safe, person-centred care. Deprivation of Liberty Safeguards (DoLS) are considered where restrictions may be required to keep people safe.<br><br>However, some inconsistencies were identified in how capacity assessments are documented. In one example, multiple decisions, such as medical treatment, finances, relationships, and environmental restrictions, were recorded within a single MCA document rather than being assessed as separate, decision-specific assessments as required by legislation. Additionally, best-interest records did not always clearly document the alternatives considered before reaching a decision. In another case, care documentation assumed a person lacked capacity at the first stage of the MCA assessment rather than evidencing the two-stage capacity test. However, the best-interest documentation did include the alternatives considered. These gaps indicate that, while staff are aware of consent processes, documentation, and the application of MCA principles, they are not always fully consistent.<br><br>Some consent forms were signed by relatives without the appropriate legal authority. In other cases, the person themselves signed despite concerns about capacity, indicating that the Mental Capacity Act was not always applied consistently. | **Action Required**<br>*Strengthen best-interest decision records to clearly demonstrate the alternatives considered and why they were not suitable.*<br><br>*Ensure the Mental Capacity Act processes are consistently applied regarding consent.* |

| | | |
|---|---|---|
| | People are generally supported to make decisions about their care and daily lives wherever possible, and staff aim to involve individuals in discussions about their support. Consent is sought in day-to-day care delivery, and people are informed about aspects of their care and support. However, improvements are required to ensure MCA documentation clearly reflects the legal framework and demonstrates that capacity is assessed for each specific decision.<br><br>In addition, some consent forms were signed by either the person's loved one and/or a staff member when no relevant legal framework was in place, e.g., a POA. The service should note that staff must not sign consent forms on behalf of residents. | **Actions Required**<br>*Review all Mental Capacity Act assessments to ensure they are decision-specific and not combined into a single document covering multiple decisions.*<br><br>*Ensure consent forms are signed only by a person with a relevant legal framework, such as a POA, when a person is assessed to lack capacity.* |

| **CARING** | **The service involves and treats people with compassion, kindness, dignity, and respect.** | | | **Good** | |
|---|---|---|---|---|---|
| | **Total possible score for CARING: 20** | | | **SCORE** | **%** |
| | | | | **14** | **70** |
| **4 = Evidence shows an exceptional standard** | **3 = Evidence shows a good standard** | **2 = Evidence shows some shortfalls** | **1 = Evidence shows significant shortfalls** | | |
| **Kindness, compassion and dignity**<br>We always treat people with kindness, empathy and compassion and we respect their privacy and dignity. We treat colleagues from other organisations with kindness and respect.<br>***Regulations 9, 10 (12)*** | | | | **Score (1 – 4): 3** | |
| | Observations and feedback from individuals indicated that staff understood their needs and supported them in a respectful and person-centred way. Staff interactions with people were calm and supportive, helping to maintain individuals' emotional well-being and sense of safety. People confirmed that staff informed them about aspects of their care and involved them in discussions where possible, which contributed to positive relationships between staff and the people they support.<br><br>Privacy and confidentiality are promoted through appropriate information sharing on a need-to-know basis and through respectful communication between staff. Staff work collaboratively with colleagues and professionals from other organisations, demonstrating respectful and professional interactions when coordinating care and support. People are supported with personal care in ways that maintain their dignity, and staff are encouraged to use appropriate language when documenting care and communicating about individuals.<br><br>The service also recognises the importance of emotional well-being, with staff providing reassurance and support during periods of distress or behavioural escalation. People are supported to maintain relationships with family members and to participate in community activities that promote social inclusion and wellbeing. | | | **No Actions Required** | |

| | | |
|---|---|---|
| | These approaches help create a caring culture where individuals feel valued and respected. | |
| **Treating people as individuals**<br>We treat people as individuals and ensure their care, support, and treatment meet their needs and preferences. We take into account their strengths, abilities, aspirations, culture, and unique backgrounds, as well as protected characteristics.<br>***Regulations 9, 10, 14, and 15*** | | **Score (1 – 4): 3** |
| | Staff aim to respect people's personal, cultural, social, and religious needs. One individual's care records identified that she prays every morning, reads her Bible, and prays before meals, demonstrating recognition of her religious practices. However, there was limited evidence that the staff team had explored supporting this individual in attending church services or engaging more widely with her faith community. This indicates that while personal religious practices are acknowledged, further opportunities to support individuals' wider cultural or spiritual aspirations could be developed.<br><br>People are encouraged to make choices about their daily routines, activities, and how they receive support. Staff communicate with individuals in ways that promote understanding and involvement in their care, ensuring people have opportunities to express their views and preferences.<br><br>Communication systems, including care plans, handovers, and communication books, support staff to understand people's individual needs and ensure consistent care**.**<br><br>People's religious and cultural diets were identified and recorded in care and support plans. Staff in another service commented that an individual also participates in his religion. | **Action Required**<br>*Where an individual practices their faith, ensure that they are supported to engage in the wider faith-based communities.* |
| **Independence, choice and control**<br>We promote people's independence, so they know their rights and have choice and control over their own care, treatment and wellbeing.<br>***Regulations 9, 12, (9A and 10)*** | | **Score (1 – 4): 2** |
| | Individuals are encouraged to maintain relationships with friends and family and are supported to engage in relationships of their choice. Staff promote access to the community and activities that support social inclusion and wellbeing. People confirmed they were informed about aspects of their care and support, which helped them feel involved in decisions about their daily lives.<br><br>Care plans generally reflect people's levels of independence and the type of support required. For example, one individual was described as largely independent and rarely sought staff support, demonstrating that the service recognises and encourages autonomy where possible. In another service, staff were observed providing hand-to-hand support to one individual to assist with mobility and confidence when moving around the environment. In another service, staff were also observed regularly informing an individual of the time, which helped reassure him, increase his | |

| | | |
|---|---|---|
| | confidence, and reduce his anxiety. These approaches demonstrate attempts to tailor support to individual needs while promoting wellbeing.<br><br>Activities and engagement opportunities are available across services, and people are supported to participate in daily routines and community activities. However, observations indicated that the promotion of independence was not always consistent. At Church Street, staff were observed undertaking some tasks with limited engagement from people using the service. At Long Lane, staff prepared meals while chatting with people, but individuals were not initially invited to participate in the preparation. Later, one person independently prepared her own lunch, demonstrating that individuals can participate in daily living tasks when opportunities are provided. These observations suggest that independence is recognised within care plans, but staff practice could more consistently encourage active participation in everyday activities. | **Actions Required**<br>*Encourage a consistent "doing with" approach rather than completing tasks on behalf of individuals where they are able to participate.*<br>*Monitor staff practice through observations and spot checks to ensure individuals are actively involved in daily routines.* |
| **Responding to people's immediate needs**<br>We listen to and understand people's needs, views and wishes. We respond to these in that moment and will act to minimise any discomfort, concern or distress.<br>***Regulations 9, 10, 11 and 12, (16)*** | | **Score (1 – 4): 3** |
| | Communication systems such as handovers, communication books, and daily records support staff to remain aware of people's needs and any changes in their well-being. Staff interactions observed during the inspection indicated that they communicated calmly with individuals and provided reassurance when required. For example, one individual who experienced anxiety was supported through reassurance and orientation to time, with staff informing him of the time regularly to help increase his confidence and reduce anxiety. This demonstrated that staff were attentive to his emotional needs and responded in ways that helped maintain his well-being.<br><br>Staff also anticipate people's needs through care planning and observation. Individuals with mobility needs were supported with appropriate equipment and assistance, including hand-to-hand support where required to ensure safety and confidence when moving around the environment. Communication between staff through handovers and documentation ensured that relevant information about people's needs was shared, enabling staff to respond promptly.<br><br>Services, such as in-house out-of-hours, emergency ambulance and NHS 111, were known by staff members. Staff understood when to use these services. | **No Actions Required** |
| **Workforce wellbeing and enablement**<br>We care about and promote the well-being of our staff, and we support and enable them to always deliver person-centred care.<br>***Regulations 9, 12, 17, 18*** | | **Score (1 – 4): 3** |

| | | |
|---|---|---|
| | The services have established whistleblowing policies that are accessible to staff and clearly outline the procedures for raising concerns. Staff demonstrated awareness of these policies and confirmed that they know how to escalate issues if needed.<br>Staff recruitment is conducted on merit, with salaries linked to the role rather than individual characteristics, ensuring fair and equitable treatment. All staff have access to annual leave, and the same benefits, and probation and induction processes are consistently applied. Flexible working arrangements appear to be supported, and there is evidence that staff wellbeing is considered, including access to counselling services and approachable management.<br><br>Staff meetings are held to review incidents, reinforce professional boundaries, uniform standards, and communication protocols, and daily delegation forms allocate tasks and record completion. | **No Actions Required** |

| **RESPONSIVE** | **The service meets people's needs** | | | **Good** | |
|---|---|---|---|---|---|
| | **Total possible score for RESPONSIVE: 28** | | | **SCORE** | **%** |
| | | | | **18** | **64** |
| **4 = Evidence shows an exceptional standard** | **3 = Evidence shows a good standard** | **2 = Evidence shows some shortfalls** | **1 = Evidence shows significant shortfalls** | | |
| **Person-centred care**<br>We make sure people are at the centre of their care and treatment choices and we decide, in partnership with them, how to respond to any relevant changes in their needs.<br>***Regulations 9, (10, 11, 12, 14)*** | | | | **Score (1 – 4): 2** | |
| | Care and support planning demonstrated a person-centred approach, with plans reflecting individual needs, preferences, and communication styles. People were involved in decisions wherever possible, including choices around daily routines, activities, and personal care.<br><br>Staff were observed supporting individuals to maintain independence in line with their capabilities, using techniques such as providing hand-over-hand support, offering choices, and giving reassurance about time or next steps to reduce anxiety. Technology, such as tablets and internet access, was available to enable engagement, and mobility equipment was provided to support independence. Key worker sessions and one-to-one interactions were used to discuss preferences, and care plans included information about cultural, religious, and personal interests, such as participation in faith practices, hobbies, and social activities.<br><br>However, there were areas for improvement:<br><br>• Some services did not consistently offer opportunities for individuals to participate in practical tasks, such as preparing meals or engaging in household routines (see Caring). | | | **No Actions Required** | |

| | | |
|---|---|---|
| | • Limited evidence was available to show that staff actively explored supporting people's aspirations, for example, attending church services or pursuing specific interests outside the home (see Caring).<br>• Documentation in care plans occasionally lacked detail on how decisions were made with the individual and did not always reflect ongoing discussions about changes in needs or preferences (see Safe and Effective). | |
| **Care provision, integration, and continuity**<br>We understand the diverse health and care needs of people and our local communities, so care is joined-up, flexible and supports choice and continuity.<br>***Regulations 9, 12, 17, (10)*** | | **Score (1 – 4): 3** |
| | Care and support provision reflected an understanding of individuals' diverse health and care needs, with support designed to be flexible and responsive. People had access to a range of health and social care services.<br><br>Care plans and risk assessments were used to guide staff in delivering consistent and safe care, and staff worked to ensure that individuals' preferences and choices were respected. Technology, such as tablets, mobile phones and internet access, supported engagement and communication. Services promoted continuity by maintaining consistent staffing wherever possible, using key worker systems, daily delegation forms, handovers, and communication books to ensure information was shared across shifts.<br><br>Where individuals moved between services or received external support, information was shared on a need-to-know basis, and people were given opportunities to visit services before moving in. Care planning reflected both health and social needs, including mobility support, health monitoring, personal care, and participation in activities. | **No Actions Required** |
| **Providing information**<br>We provide appropriate, accurate, and up-to-date information in formats tailored to individual needs.<br>***Regulations 9, 13, 17*** | | **Score (1 – 4): 2** |
| | Information was provided to people in a range of formats to support accessibility and understanding. Paper-based care plans, profiles, and "About Me" documents were available, with some large-font formats for those with visual needs. People had access to technology such as tablets and the internet, which supported communication and engagement. Staff demonstrated awareness of individuals' communication preferences, using short sentences, simple language, and Makaton where required. Staff were observed communicating with clients who prefer this method.<br>Tenancy agreements and consent forms were generally provided. However, some documents were not in user-friendly or pictorial formats, and there was limited evidence that explanations had been consistently shared with service users and explained in ways they could fully understand. Staff shared relevant information on a | **Action Required**<br>*Review and update all service documents, including tenancy agreements, consent forms, and service guides, to ensure they are* |

| | | |
|---|---|---|
| | need-to-know basis, ensuring privacy and confidentiality were maintained.<br><br>GDPR compliance was observed in terms of recording and storing personal information.<br><br>Gaps were identified in ensuring full transparency and accessibility: some individuals had consent forms signed only by relatives without proper legal authority, and evidence of service users being fully informed about fees, rights, or changes to their care was minimal (see Effective).<br><br>In one service, staff members were observed speaking to an individual in her mother tongue, which enabled the person to understand the information being shared. | *accessible, user-friendly, and, where required, in pictorial versions.* |
| **Listening to and involving people**<br>We make it easy for people to share feedback and ideas or raise complaints about their care, treatment and support. We involve them in decisions about their care and tell them what's changed as a result.<br>***Regulations 16, 17 (9, 10)*** | | **Score (1 – 4): 3** |
| | Surveys were conducted for service users, staff, and family and friends to gather feedback on care, support, and working conditions. The feedback generally showed that service users were content with the care and support they received, families were happy with the care provided and the support offered to their loved ones, and staff reported satisfaction with their work environment and felt supported in their roles.<br><br>Voices meetings complemented these surveys, providing individuals with an opportunity to express preferences, share their experiences, and communicate what matters to them. Minutes from February 2026 show people enjoyed activities, maintained contact with loved ones, and were satisfied with staff interactions. User-friendly meeting minutes and records of preferred communication methods supported accessibility and understanding. | **No Actions Required** |
| **Equity in access**<br>We make sure that everyone can access the care, support and treatment they need when they need it.<br>***Regulations 12, 13,15 17, (9, 10)*** | | **Score (1 – 4): 3** |
| | People were generally able to access care, support, and treatment in line with their needs, with reasonable adjustments made for mobility, communication, and health conditions.<br><br>Each service was in the community and had access to public transport and local communities. People were supported in decorating their rooms as they wished and bringing artefacts of their choice.<br><br>Emergency and unplanned care arrangements were in place, including on-call staff, GP and dentist access, and support to attend health appointments. Some waiting times for clinical interventions | **No Actions Required** |

| | | |
|---|---|---|
| | or specialist referrals were noted, and these were communicated to service users to reduce anxiety. Staff provided support during appointments and followed care plans to maintain continuity of care. | |
| **Equity in experiences and outcomes**<br>We actively seek out and listen to information about people who are most likely to experience inequality in experience or outcomes. We tailor the care, support and treatment in response to this.<br>***Regulations 12, 13, 17, (9, 10)*** | | **Score (1 – 4): 2** |
| | There were processes in place to understand and respond to people at risk of experiencing inequality in care, support, and outcomes. Surveys were conducted with service users, staff, and family members, and feedback generally showed that individuals were content with the care and support they received, families were satisfied with services, and staff reported positive experiences at work. Voices, meetings and user-friendly minutes enabled people to express preferences, participate in decisions, and share experiences.<br><br>Some barriers were identified, including limited engagement in activities where staff completed tasks without fully involving individuals, inconsistent support for religious or cultural practices, delays in access to specialist referrals, and occasional gaps in communication for people with limited literacy or digital skills (see Safe, Effective and Caring).<br><br>Staff were observed providing tailored support for individuals who required hand-to-hand assistance or reassurance to reduce anxiety, and mobility aids and technology were available to promote independence. | **No Actions Required** |
| **Planning for the future**<br>We support people in planning for important life changes so they have enough time to make informed decisions about their future, including at the end of their life.<br>***Regulations 9, 10, (11)*** | | **Score (1 – 4): 2** |
| | Services demonstrate limited evidence of supporting people to plan for important life changes, including end-of-life care. While some documents, such as health passports and medical histories, are in place, there is little evidence that advance care planning, DNACPR/ ReSPECT discussions, or palliative care planning have been completed with individuals.<br><br>Staff liaise with families where appropriate, but the lack of formal discussions or documented decisions means that people's preferences for future care, including at the end of life, are not consistently recorded or fully understood. | **Action Required**<br>*Develop and implement a structured forward-planning framework that ensures all individuals have documented discussions about future care, including DNACPR/ReSPECT, palliative care, and end-of-life preferences, with regular reviews involving the person, their family, and relevant professionals.* |

| WELL-LED | Leadership, management and governance of the organisation assure the delivery of high-quality and person-centred care, support learning and innovation, and promote an open and fair culture. | Requires Improvement | |
|---|---|---|---|
| | Total possible score for WELL-LED: 28 (excludes environmental sustainability): | SCORE | % |
| | | 17 | 61 |

| 4 = Evidence shows an exceptional standard | 3 = Evidence shows a good standard | 2 = Evidence shows some shortfalls | 1 = Evidence shows significant shortfalls |
|---|---|---|---|

| | | |
|---|---|---|
| **Shared direction and culture**<br>We have a shared vision, strategy and culture. This is based on transparency, equity, equality and human rights, diversity and inclusion, engagement, and understanding the challenges and needs of people and our communities to meet them.<br>***Regulations 10, 12, 17, (9) Related: 12*** | | **Score (1 – 4): 2** |
| | Policies, such as the business continuity plan, reflect emergency planning, while infection control, safety, and safeguarding frameworks reinforce a culture of responsibility and accountability. Staff were observed to follow consistent procedures, communicate positively with one another, and work collaboratively across teams, demonstrating commitment to shared values and person-centred care.<br><br>The organisation also recognises its wider social responsibilities by supporting residents to access community activities, maintain relationships, and have opportunities for independence, demonstrating an approach that considers the broader impact on the lives of people using services. However, records did not always reflect community engagement, but in-house/indoor activities (see Caring). | **No Actions Required** |
| **Capable, compassionate and inclusive leaders**<br>We have inclusive leaders at all levels who understand the context in which we deliver care, treatment and support and embody the culture and values of their workforce and organisation. They have the skills, knowledge, experience and credibility to lead effectively. They do so with integrity, openness and honesty.<br>***Regulations 6, 7, 18, 19 (4, 5) Related: 4, 14*** | | **Score (1 – 4): 3** |
| | Policies such as the business continuity plan reflect planning for emergencies, while infection control, safety, and safeguarding frameworks reinforce a culture of responsibility and accountability. Staff were observed to follow consistent procedures, communicate positively with each other, and work collaboratively across different teams, showing commitment to shared values and person-centred care.<br><br>The organisation also recognises its wider social responsibilities by supporting residents to access community activities, maintain relationships, and have opportunities for independence, demonstrating an approach that considers the broader impact on the lives of people using services. However, records did not always reflect community engagement, but in-house/indoor activities (see Caring).<br><br>Staff spoken with commented that they had received adequate training. | **No Actions Required** |

| | | |
|---|---|---|
| | Staff supervisions had taken place, and staff reported that they were supported, had supervision and probation. One staff member commented, "*I have worked in care before, but this company is very good. I am happy at work, the money is good, and I enjoy my work, and I can't ask for more".* | |
| **Freedom to speak up**<br>We foster a positive culture where people feel that they can speak up and that their voice will be heard.<br>***Regulations 10, 12, 17 (9)*** | | **Score (1 – 4): 2** |
| | The services have established whistleblowing policies that are accessible to staff and clearly outline the procedures for raising concerns. Staff demonstrated awareness of these policies and confirmed that they know how to escalate issues if needed. However, there is no appointed Freedom to Speak Up Guardian, which limits an independent channel for raising concerns. While staff feel able to report issues within the line management structure, the absence of a dedicated guardian may reduce confidence for those who prefer to speak up externally.<br>There is no evidence of a closed culture. | **Action Required**<br>*Appoint a Freedom to Speak Up Guardian across all services and ensure staff are informed of their role, how to access them, and the process for raising concerns independently of line management.* |
| **Workforce equality, diversity and inclusion**<br>We value diversity in our workforce. We work towards an inclusive and fair culture by improving equality and equity for people who work for us.<br>***Regulations 17, 18*** | | **Score (1 – 4): 3** |
| | Staff recruitment is conducted on merit, with salaries linked to the role rather than individual characteristics, ensuring fair and equitable treatment. All staff have access to annual leave, and the same benefits, and probation and induction processes are consistently applied. Flexible working arrangements appear to be supported, and there is evidence that staff wellbeing is considered, including access to counselling services and approachable management. | **No Actions Required** |
| **Governance, management and sustainability**<br>We have clear responsibilities, roles, systems of accountability and good governance. We use these to manage and deliver good quality, sustainable care, treatment and support. We act on the best information about risk, performance and outcomes, and we share this securely with others when appropriate.<br>***Regulations 17, (12) Related: 14, 15, 16, 18, 20, 22A*** | | **Score (1 – 4): 2** |
| | There is a system for governance, management, and sustainability, with clear roles and responsibilities identified across most areas, although there are gaps in documentation and oversight in some services.<br><br>At Chase Road, in-house audits were not available at the time of inspection, with the area manager confirming that all records are held at the head office. This limited the ability to fully assess local audit processes and the implementation of corrective actions on-site. Staff were aware of their roles and responsibilities, but the absence of on-site documentation highlights a gap in accessible governance oversight. | **Action Required**<br>*Ensure managers have access to audits for their services.* |

| | | |
|---|---|---|
| | At 52 Long Lane, in-house audits are conducted routinely across key areas. Weekly medication audits identified instances where a prescribed toothpaste was administered incorrectly, and expired products, including olive oil ear drops and eye drops with incorrect expiry dates, were noted. These were addressed, but documentation did not consistently record actions taken. Health and safety audits highlighted areas where floors had not been adequately cleaned, and infection prevention and control audits revealed that bedroom and laundry areas were not cleaned as scheduled, and waste management policies were not fully understood by staff. Monthly medication audits are completed, but details are limited in scope and require greater specificity to fully capture compliance and corrective actions.<br><br>The service has systems in place to manage accountability, quality assurance, and governance, with clear structures for staff responsibilities. Emergency preparedness, including for climate-related events, is recognised, although records show that some plans require more detail. Cybersecurity and data protection are considered through the DSPT, and workforce planning is evident, though not all services use digital systems. For example, some services were paper-based and did not use Cura Care planning software.<br><br>Statutory and regulatory compliance is monitored, and safety alerts and external recommendations are addressed, but the effectiveness of follow-up actions varies between sites. Sustainability, including financial and workforce considerations, is factored into planning, although some gaps in local oversight were noted.<br><br>Renovations/wooden floor was being installed; however, there was no risk assessment demonstrating how risks such as dust, trip/falls, and equipment falling on people were being mitigated (see Safe)**.** | **Action Required**<br>*Ensure auditing documentation clearly records outcomes from addressing any findings.*<br><br>**Actions Required**<br>*Standardise digital records across all services, ensuring Cura Care software is consistently used and staff are trained on its use.*<br><br>*Ensure external safety alerts and recommendations are logged, communicated to relevant staff, and acted on promptly.* |
| **Partnerships and communities**<br>We understand our duty to collaborate and work in partnership, so our services work seamlessly for people. We share information and learning with partners and collaborate for improvement.<br>***Regulations 12, 17 (9)*** | | **Score (1 – 4): 3** |
| | Individuals are supported to access healthcare professionals such as GPs, dentists, and other specialists to maintain their physical and mental health.<br><br>Outcomes are also supported through access to healthcare professionals, such as GPs, psychiatrists, psychologists, physiotherapists, and occupational therapists. Staff monitor people for changes in health or behaviour and escalate concerns where required.<br><br>People are encouraged to take part in daily activities and community engagement that support physical movement and | **No Actions Required** |

| | | |
|---|---|---|
| | social interaction, helping maintain both physical and emotional health. However, for one individual, there was limited evidence that the staff team had explored supporting them to attend church services or engage more widely with her faith community. This indicates that while personal religious practices are acknowledged, further opportunities to support individuals' wider cultural or spiritual aspirations through working partnerships with other services could be developed (see Caring). | |
| **Learning, improvement and innovation**<br>We focus on continuous learning, innovation and improvement across our organisation and the local system. We encourage creative ways of delivering equality of experience, outcome and quality of life for people. We actively contribute to safe, effective practice and research.<br>***Regulation 17 (16)*** | | **Score (1 – 4): 2** |
| | The service completes audits, competency assessments, and regular supervision to identify areas for development. For example, MAR chart audits, IPC audits, and health and safety checks have highlighted areas needing attention, and actions have been taken to address some of these findings, such as returning expired medication and addressing cleanliness issues.<br><br>Innovation in practice is emerging, with the use of digital care-planning systems, such as Cura software, in some services. However, this is not fully implemented across all locations, as previously mentioned.<br><br>Staff have opportunities to learn from incidents, complaints, and service user feedback, and surveys conducted with service users, staff, and families provide valuable insights into the quality of care and wellbeing outcomes.<br><br>A risk register was not seen, and an example template was shared with the manager.<br><br>Learning from deaths and serious incidents is acknowledged as an area requiring formalisation. While individual events may be reviewed, there is limited structured evidence of systematic analysis, shared learning, and integration of findings across all services. Research and evidence-based practice are referenced in some care plans, though there is scope to use research findings to guide innovation and improvement consistently. | **Actions Required**<br>*Develop and implement a risk register.*<br><br>*Encourage staff to engage with research and evidence-based practice, including relevant training and workshops.* |
| **Environmental sustainability – sustainable development**<br>We understand any negative impact of our activities on the environment and we strive to make a positive contribution to reducing it and support people to do the same.<br>***Regulation 17*** | | **Score (1 – 4): Not scored** |
| | This quality statement is not currently being scored by the CQC. | |

**END of Report**